

Elacestrant plus abemaciclib (abema) combination in patients (pts) with estrogen receptor-positive (ER+), HER2-negative (HER2-) advanced or metastatic breast cancer (mBC)

Presented at NASP 2025 Annual Meeting & Expo
September 14-17, 2025

This poster was originally presented at SABCS 2024.

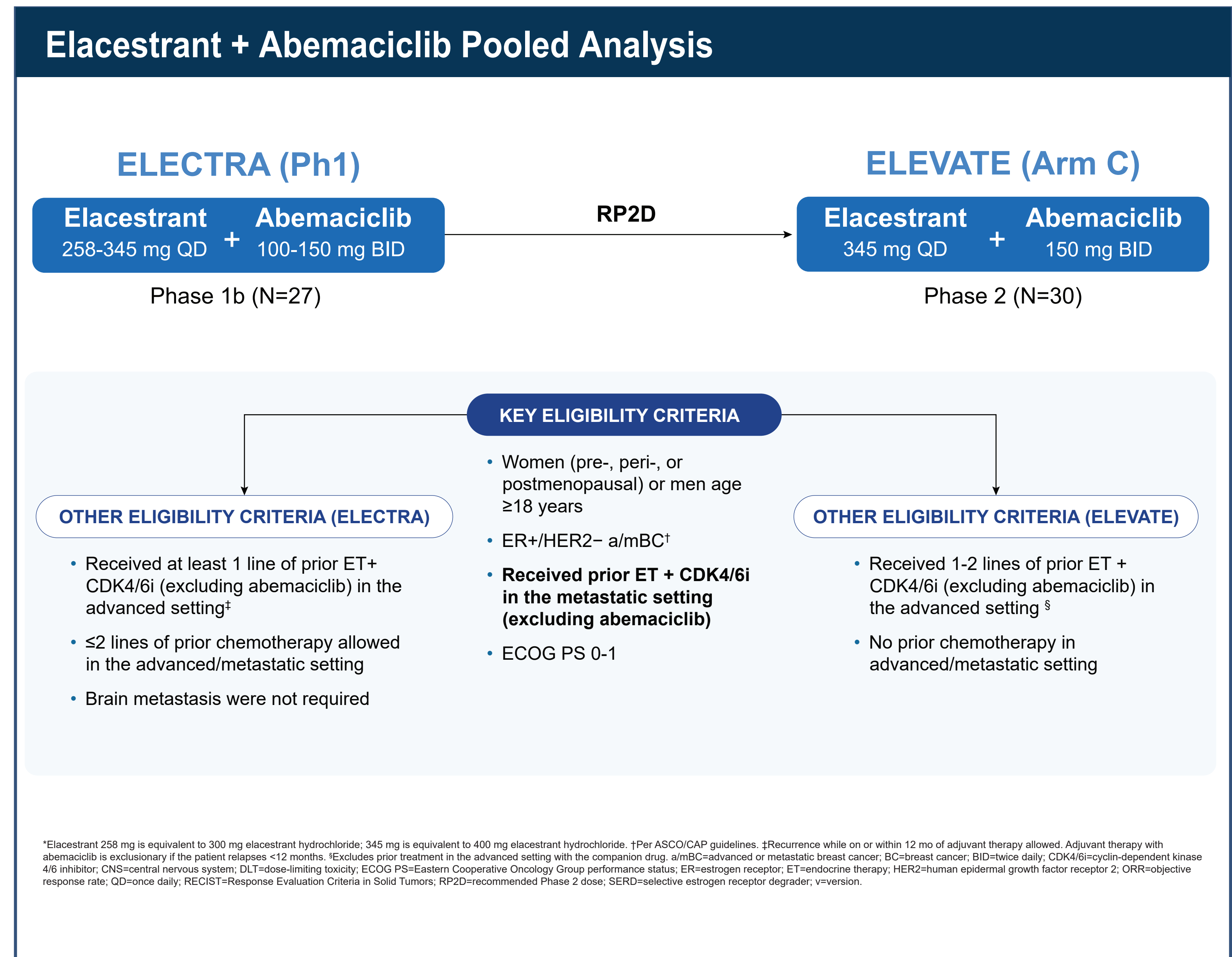
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BACKGROUND

- After first-line endocrine therapy (ET) + CDK4/6i in patients ER+/HER2- mBC, tumors eventually develop resistance to intrinsic alterations in the cell cycle or PI3K/AKT/mTOR pathways, or to acquired *ESR1*-mutations that emerge in up to 50% of patients, leading to disease progression.¹⁻¹⁸
- In the phase 3 EMERALD trial, single-agent elacestrant significantly prolonged mPFS with a manageable safety profile vs. SOC ET, leading to the first oral SERD approved in ER+/HER2- mBC that targets *ESR1*-mutated tumors.^{19,20}
 - Patients with *ESR1*-mutated tumors had a 45% reduction in risk of progression or death with elacestrant vs SOC (HR=0.55; 95% CI: 0.39–0.77; *P*=0.0005).²⁰
 - In patients with ≥12 months of prior ET + CDK4/6i and *ESR1*-mutated tumors, mPFS with elacestrant was 8.6 months vs 1.9 months with SOC (HR=0.41; 95% CI: 0.26–0.63).²¹
- The rationale for combining elacestrant + abemaciclib is to overcome different resistance mechanisms and enable an all-oral regimen.
- In the ELECTRA phase 1b study (NCT05386108), the recommended phase 2 dose (RP2D) was determined:²²
 - Elacestrant 345 mg + abemaciclib 150 mg BID (RP2D)
 - ORR=25%; CBR at 16 weeks = 75%
 - PK analyses showed no significant drug-drug interactions with a manageable safety profile
- This pooled analysis reports updated safety and preliminary efficacy with the elacestrant + abemaciclib combination in patients with prior ET + CDK4/6i exposure (excluding abemaciclib) from the ongoing ELECTRA (phase 1b portion) and ELEVATE (phase 2 [Arm C] NCT05563220) studies.

METHODS



RESULTS

Baseline Characteristics					
	ELECTRA Cohort 1 (n=8)	ELECTRA Cohort 2 (n=7)	ELECTRA Cohort 3* RP2D (n=12)	ELEVATE Arm C (n=30)	POOLED ANALYSIS ELECTRA Cohort 3* + ELEVATE Arm C (n=42)
Parameters	Elacestrant 258 mg QD + Abemaciclib 100 mg BID	Elacestrant 345 mg QD + Abemaciclib 100 mg BID	Elacestrant 345 mg QD + Abemaciclib 150 mg BID	Elacestrant 345 mg QD + Abemaciclib 150 mg BID	Elacestrant 345 mg QD + Abemaciclib 150 mg BID
Median age, years (range)	43 (32–67)	51 (41–71)	54 (48–74)	61 (29–84)	60 (29–84)
Female, n (%)	8 (100)	7 (100)	12 (100)	30 (100)	42 (100)
ECOG PS, n (%)					
0	5 (63)	4 (57)	7 (58)	20 (67)	27 (64)
1	3 (38)	3 (43)	5 (42)	10 (33)	15 (36)
Metastatic site, n (%)					
Visceral	6 (75)	6 (86)	8 (67)	22 (73)	30 (71)
Brain	0	0	0	0	0
Liver	5 (63)	3 (43)	8 (67)	9 (30)	17 (41)
Lung	1 (13)	5 (71)	3 (25)	8 (27)	11 (26)
Bone	5 (63)	5 (71)	6 (50)	14 (47)	20 (48)
Mutations, ¹ n (%)					
<i>ESR1</i>	2/5 (40)	2/7 (29)	7/12 (58)	14/28 (50)	21/40 (53)
<i>PIK3CA</i>	1/5 (20)	3/7 (43)	2/12 (17)	9/28 (32)	11/40 (28)
Primary endocrine resistance, ² n (%)	3 (38)	1 (14)	3 (25)	2 (7)	5 (12)
Median number of prior therapies for adv/mBC, n (range)	2 (1–3)	2 (1–4)	2 (1–6)	1 (1–2)	1 (1–6)
Prior CDK4/6i for adv/mBC, n (%)	5 (63)	7 (100)	12 (100)	30 (100)	42 (100)
Palbociclib	3 (38)	5 (71)	5 (42)	18 (60)	23 (55)
Ribociclib	2 (25)	2 (29)	6 (50)	11 (37)	17 (40)
Palbociclib/Ribociclib	0	0	1 (8)	1 (3)	2 (5)
Number of prior lines of endocrine therapy for adv/mBC, n (%)					
1	3 (38)	3 (43)	5 (42)	25 (83)	30 (71)
2	3 (38)	3 (43)	6 (50)	5 (17)	6 (14)
3	0	1 (14)	1 (8)	0	1 (2)
Type of prior endocrine therapy, n (%)					
Fulvestrant	5 (63)	5 (71)	9 (75)	13 (43)	22 (52)
AI	3 (38)	5 (71)	9 (75)	21 (70)	30 (71)
Tamoxifen	1 (13)	2 (29)	2 (17)	1 (3)	3 (7)
Tamoxifen/AI	0	1 (14)	1 (8)	1 (3)	2 (5)
Number of prior lines of chemotherapy, n (%)					
0	5 (63)	3 (43)	5 (42)	30 (100)	35 (83)
1	3 (38)	4 (57)	6 (50)	0	6 (14)
2	0	0	1 (8)	0	1 (2)

Data cut-off: 15 OCT 2024. *Includes confirmatory cohort 3 expansion. ¹Mutation data not available for all patients at time of data analysis. ²Relapse within the first two years while on adjuvant ET and/or progressive disease within the first six months of baseline ET for advanced/metastatic breast cancer. ³adv/mBC=advanced or metastatic breast cancer; AI=aromatase inhibitor; BID=twice daily; CDK4/6i=cyclin-dependent kinase 4/6 inhibitor; ECOG PS=Eastern Cooperative Oncology Group performance status; ET=endocrine therapy; QD=once daily; RP2D=recommended phase 2 dose.

Treatment-Emergent Adverse Events (TEAEs) ≥20% at RP2D					
	ELECTRA Cohort 1 (n=8)	ELECTRA Cohort 2 (n=7)	ELECTRA Cohort 3* RP2D (n=12)	ELEVATE Arm C (n=30)	POOLED ANALYSIS ELECTRA Cohort 3* + ELEVATE Arm C (n=42)
	Elacestrant 258 mg QD + Abemaciclib 100 mg BID	Elacestrant 345 mg QD + Abemaciclib 100 mg BID	Elacestrant 345 mg QD + Abemaciclib 150 mg BID	Elacestrant 345 mg QD + Abemaciclib 150 mg BID	Elacestrant 345 mg QD + Abemaciclib 150 mg BID
Preferred Term, n (%)	All Grades Grade 3	All Grades Grade 3	All Grades Grade 3	All Grades Grade 3	All Grades Grade 3
Diarrhea	5 (63) 0	6 (86) 0	11 (92) 0	24 (80) 2 (7)	35 (83) 2 (5)
Nausea	6 (75) 0	5 (71) 0	8 (67) 0	19 (63) 2 (7)	27 (64) 2 (5)
Vomiting	1 (13) 0	3 (43) 0	4 (33) 1 (8)	13 (43) 0	17 (41) 1 (2)
Fatigue	1 (13) 0	1 (14) 0	2 (17) 0	13 (43) 2 (7)	15 (36) 2 (5)
Neutropenia/neutrophil decreased	2 (25) 2 (25)	3 (43) 2 (29)	9 (75) 7 (58)	5 (17) 4 (13)	14 (33) 11 (26)
Anemia	1 (13) 0	2 (29) 0	4 (33) 1 (8)	6 (20) 2 (7)	10 (24) 3 (7)
Constipation	0 0	3 (43) 0	0 0	9 (30) 0	9 (21) 0
Decreased appetite	2 (25) 0	3 (43) 0	2 (17) 0	7 (23) 0	9 (21) 0

No grade 4 AEs were reported during the elacestrant + abemaciclib treatment period

Data cut-off: 15 OCT 2024. *Includes confirmatory cohort 3 expansion. AE=adverse event; BID=twice daily; CDK4/6i=cyclin-dependent kinase 4/6 inhibitor; ET=endocrine therapy; QD=once daily; RP2D=Recommended Phase 2 dose.

Response and Clinical Benefit With Elacestrant + Abemaciclib in Efficacy-Evaluable Patients

	ELECTRA Cohort 1 (n=7)	ELECTRA Cohort 2 (n=7)	ELECTRA Cohort 3* RP2D (n=12)	ELEVATE Arm C (n=26)	POOLED ANALYSIS ELECTRA Cohort 3* + ELEVATE Arm C (n=38)
	Elacestrant 258 mg QD + Abemaciclib 100 mg BID	Elacestrant 345 mg QD + Abemaciclib 100 mg BID	Elacestrant 345 mg QD + Abemaciclib 150 mg BID	Elacestrant 345 mg QD + Abemaciclib 150 mg BID	Elacestrant 345 mg QD + Abemaciclib 150 mg BID
Efficacy Outcome**					
ORR, n (%)	2 (29)	2 (29)	3 (25)	4 (15)	7 (18)
CR	-	-	1 (8)	1 (4)	2 (5)
PR	2 (29)	2 (29)	2 (17)	3 (12)	5 (13)
SD	2 (29)	3 (43)	7 (58)	18 (69)	25 (66)
PD	3 (43)	2 (29)	2 (17)	4 (15)	6 (16)
CBR, n (%)	4 (57)	5 (71)	10 (83)	22 (85)	32 (84)
CBR24wks, n (%)	4 (57)	4 (57)	8 (67)	In ELEVATE Arm C, average observation time has not reached 24 weeks	

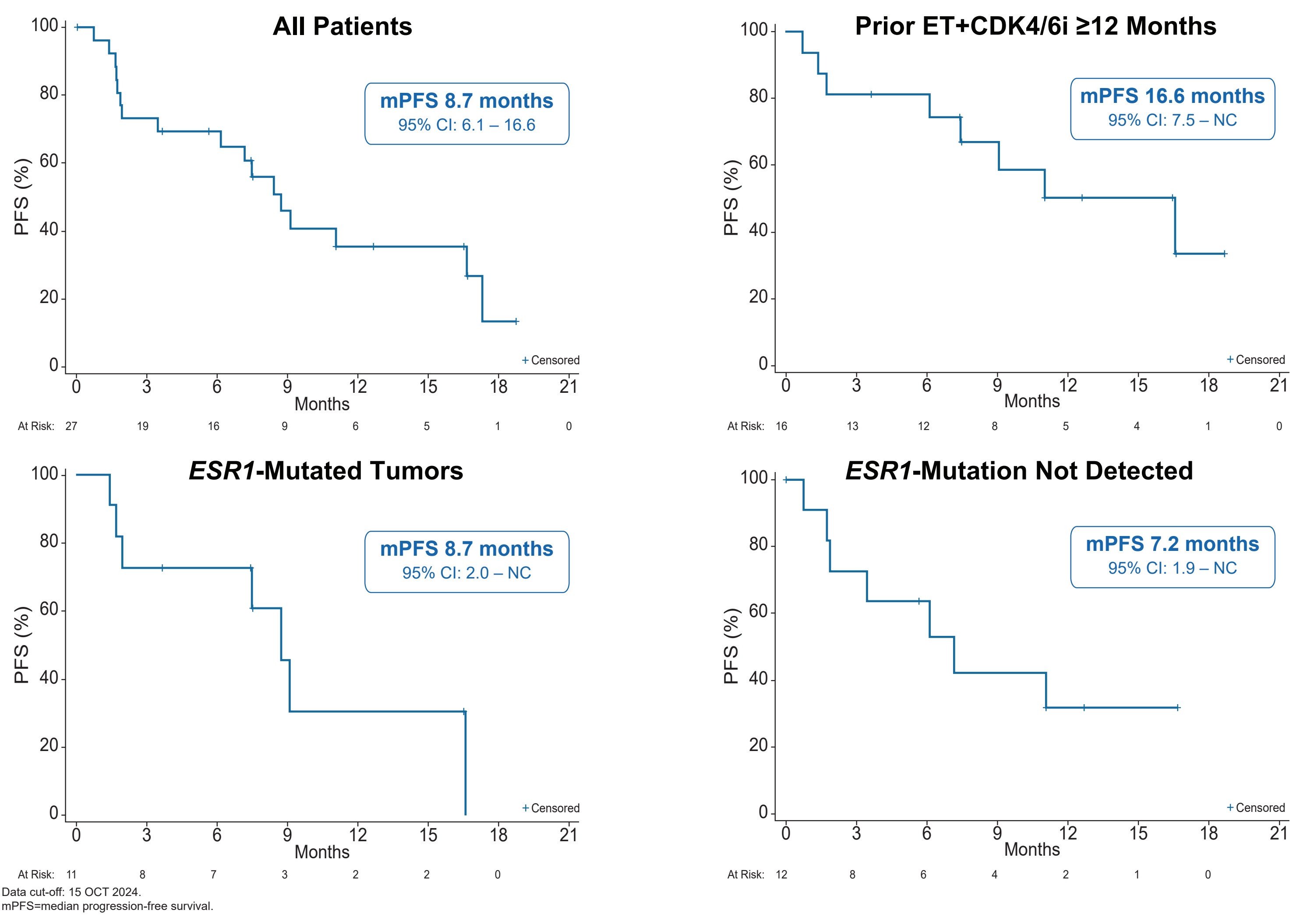
Data cut-off: 15 OCT 2024. *Confirmed responses only. [†]Includes patients who had measurable disease (ie, at least 1 target lesion) at baseline and at least 1 post-baseline RECIST assessment. [‡]Includes confirmatory cohort 3 expansion. CBR=clinical benefit rate [CR + PR + SD]; CR=complete response; PR=partial response; SD=stable disease; CDK4/6i=cyclin-dependent kinase 4/6 inhibitor; ET=endocrine therapy; CBR=objective response rate; RECIST=Response Evaluation Criteria in Solid Tumors.

mPFS in Efficacy-Evaluable Patients from ELECTRA Phase 1b

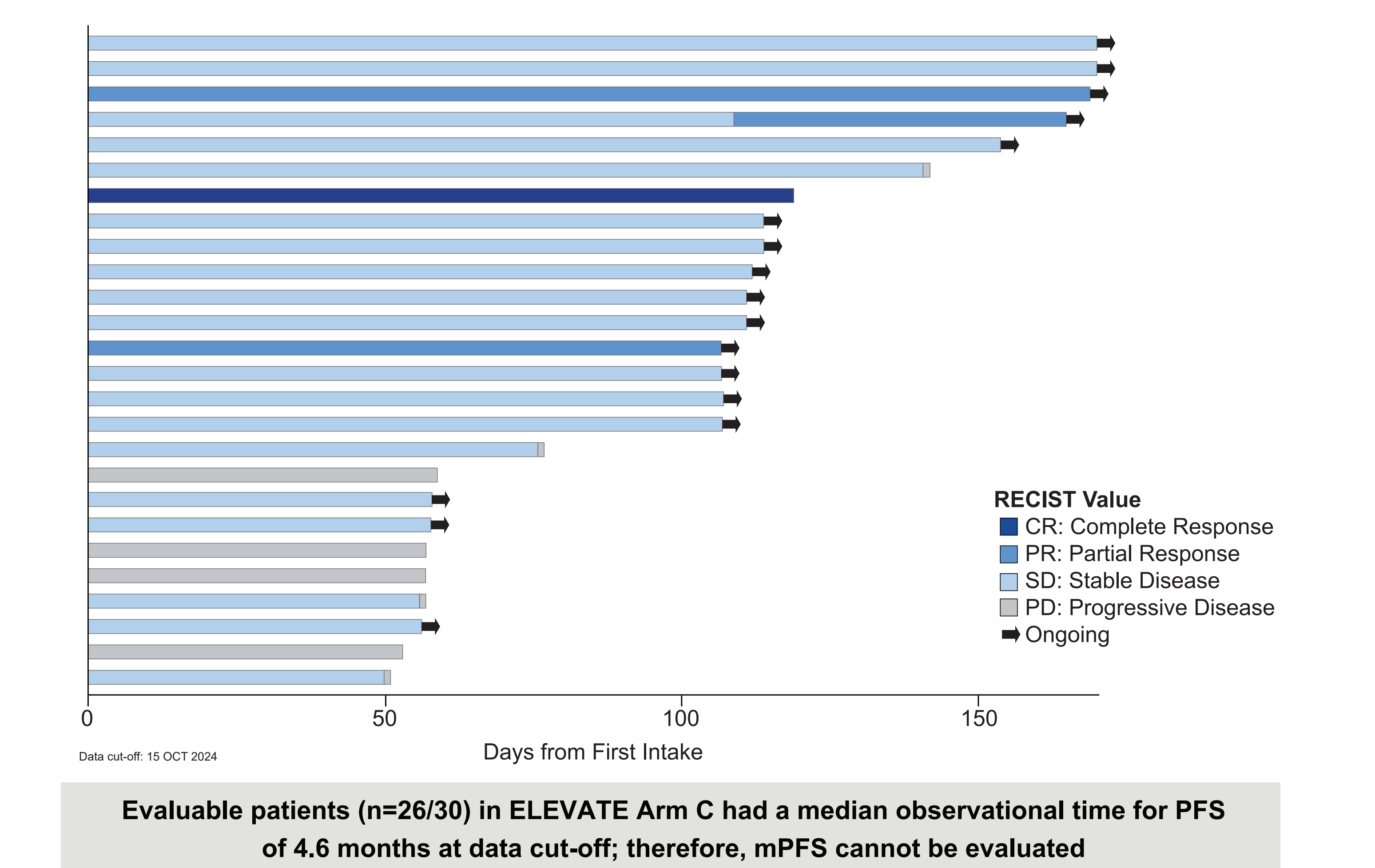
Elacestrant + Abemaciclib			
Population	n	mPFS, mo	[95% CI]
All patients	27	8.7	[6.1 – 16.6]
Prior ET+CDK4/6i	24	8.7	[6.1 – 16.6]
<i>ESR1</i> -mutated tumors	27	8.7	[2.0 – NC]
<i>ESR1</i> -mutation not detected	12	7.2	[1.9 – NC]
Prior ET+CDK4/6i ≥12 months	16	16.6	[7.5 – NC]
Dose level			
Elacestrant 345 mg QD + Abemaciclib 150 mg BID (RP2D)	12	8.7	[7.2 – NC]
Elacestrant 345 mg QD + Abemaciclib 100 mg BID	7	7.5	[1.9 – NC]
Elacestrant 258 mg QD + Abemaciclib 100 mg BID	8	8.4	[1.7 – 17.3]

Patients in ELECTRA Phase 1b had a median observational time for PFS of 7.5 months at data cut-off

- Clinical characteristics: visceral metastases 74%, prior fulvestrant 70%, ≥2 prior lines of ET for adv/mBC 52%, primary endocrine resistance 26%, *ESR1*-mutated tumors 46% (11/24) and *PIK3CA*-mutated tumors 25% (6/24)



ELEVATE Arm C: Best Tumor Response



Valuable patients (n=26/30) in ELEVATE Arm C had a median observational time for PFS of 4.6 months at data cut-off; therefore, mPFS cannot be evaluated

- Clinical characteristics for patients in ELEVATE Arm C: visceral metastases 73%, prior fulvestrant 43%, two prior lines of ET for adv/mBC 17%, primary endocrine resistance 7%, *ESR1*-mutated tumors 50% (14/28), and *PIK3CA*-mutated tumors 32% (9/28).

CONCLUSIONS

- In patients with prior ET + CDK4/6i, the safety of elacestrant + abemaciclib was consistent with the known profile of abemaciclib + standard ET, and no new signals were observed.
 - No grade 4 AEs were observed during the treatment period.
- Elacestrant + abemaciclib showed clinically important efficacy regardless of *ESR1*-mutation status.
 - mPFS was 8.7 months in all patients.
 - mPFS was 8.7 months in patients with *ESR1*-mutated tumors and 7.2 months in tumors with *ESR1*-mutation not detected.
 - mPFS was 16.6 months in those patients who received prior ET + CDK4/6i ≥12 months.
- The phase 2 portion of the combination of elacestrant 345 mg QD + abemaciclib 150 mg BID is currently ongoing.
- Elacestrant could become an ET backbone in combination with abemaciclib in patients with ER+/HER2- mBC due to its potential to extend PFS, enable an all-oral treatment option, and delay chemotherapy or ADC-based regimens.

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